

2024, Mar. 2024 Release LOC:Acute Adult

Arrhythmia, Blocks

Utilization Benchmarks: *Multiple options available, see table below for options*

Overview

Select Day

Initial review ⁽¹⁾

Episode Day 1

Episode Day 2

Episode Day 3

Episode Day 4

Discharge Screens ⁽³⁶⁾

Utilization Benchmarks

Notes

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Overview

Instruction: This subset is for patients who present with heart block and require evaluation and treatment. For atrial arrhythmias see the Arrhythmia: Atrial subset. For ventricular arrhythmias see the Arrhythmia: Ventricular or abnormal electrocardiogram (ECG) finding subset. Criteria for the following heart blocks are found within this subset:

- Junctional escape rhythm
- Accelerated junctional escape rhythm
- 2nd degree heart block (Mobitz type II)
- 3rd degree heart block

Introduction:

Arrhythmias are often triggered by alcohol, myocardial infarction (MI), pericarditis, myocarditis, pulmonary embolism, pulmonary disease, hyperthyroidism, metabolic disease, electrolyte imbalance, or cardiac drug toxicity. Presenting symptoms of an arrhythmia often include: palpitations, chest pain, dyspnea, fatigue, lightheadedness, or syncope.

Evaluation/Intervention:

Patients presenting with an arrhythmia should undergo a thorough history and physical examination. A 12-lead ECG is recommended in all patients being evaluated for an arrhythmia. Additional testing may be necessary and includes: left ventricular functional assessment using echocardiogram, computed tomography (CT), cardiac magnetic resonance imaging (CMR), electrophysiology study (EPS), and laboratory testing.

Treatment goals for patients with an arrhythmia may include restoration of sinus rhythm, rate control, prevention of thromboembolism, and the prevention of future arrhythmias. Management may include discontinuation of proarrhythmic medication, initiation of antiarrhythmic medication, immunotherapy with digoxin specific antibody, insertion of an implantable cardioverter defibrillator (ICD) or pacemaker, ablation, or surgery (Kusumoto et al., Circulation 2019, 140: e382-e482).

InterQual® criteria are derived from the systematic, continuous review and critical appraisal of the most current evidence-based literature and include input from our independent panel of clinical experts. To generate the most appropriate recommendations, a comprehensive literature review of the clinical evidence was conducted. Sources searched included PubMed, Agency for Healthcare Research and Quality (AHRQ) Comparative Effectiveness Reviews, National Institute for Health and Care Excellence (NICE), the Centers for Medicare and Medicaid Services (CMS), The Cochrane Library, and The Joint Commission. Other medical literature databases, medical content providers, data sources, regulatory body websites, and specialty society resources may also have been utilized. Relevant studies were assessed for risk of bias following principles described in the *Cochrane Handbook*. The resulting evidence was assessed

for consistency, directness, precision, effect size, and publication bias. Observational trials were also evaluated for the presence of a dose-response gradient and the likely effect of plausible confounders. The content is based on a variety of references which are cited at specific criteria points throughout the subset.

(Excludes PO medications unless noted)

Select Day, One:● **Initial review, One:** ⁽¹⁾

(From arrival in ED through decision to admit)

(Symptom or finding within 24h)

● **INTERMEDIATE, One:** ⁽²⁾● Junctional rhythm or atrioventricular block, ≥ **One:** ^(3, 4)

- Accelerated or junctional escape rhythm and systolic blood pressure ≥ 90 mmHg and symptomatic ^(5, 6, 7)
- 2nd degree heart block (Mobitz II) ^(8, 9)

● **CRITICAL, One:** ⁽¹⁰⁾● Junctional rhythm or atrioventricular block, ≥ **One:** ⁽³⁾

- Documented pause ≥ 3 sec
- Accelerated or junctional escape rhythm and systolic blood pressure < 90 mmHg ^(5, 11)
- 2nd degree heart block (Mobitz II) and systolic blood pressure < 90 mmHg ⁽⁹⁾
- 3rd degree heart block ⁽¹²⁾

● **Episode Day 1, One:**

(Symptom or finding within 24h)

● **INTERMEDIATE, ≥ One:** ⁽²⁾

- Fractured or displaced pacemaker lead (use **General Medical** subset)
- Implantable device failure (use **General Medical** subset)

● Junctional rhythm or atrioventricular block, **Both:** ^(3, 4)● Finding, ≥ **One:**

- Accelerated or junctional escape rhythm and systolic blood pressure ≥ 90 mmHg and symptomatic ^(5, 6)
- 2nd degree heart block (Mobitz II)

● Intervention, ≥ **One:**

- Atropine
- Permanent pacemaker placement ≤ 24h ^(13, 14)
- Suspected drug toxicity and 2nd degree heart block (Mobitz II) requiring continuous cardiac monitoring (excludes Holter) ^(3, 9)

● **CRITICAL, ≥ One:** ⁽¹⁰⁾

- Hemodynamic monitoring, invasive ⁽¹⁵⁾

● Junctional rhythm or atrioventricular block, **Both:** ⁽³⁾● Finding, ≥ **One:**

- Documented pause ≥ 3 sec
- Accelerated or junctional escape rhythm and systolic blood pressure < 90 mmHg ^(5, 11)
- 2nd degree heart block (Mobitz II) and systolic blood pressure < 90 mmHg ⁽⁸⁾
- 3rd degree heart block ⁽¹²⁾

● Intervention, ≥ **One:**

- Atropine

● Beta blocker toxicity and, ≥ **One:** ⁽¹⁶⁾

- Glucagon
- High dose insulin ⁽¹⁷⁾

● Calcium channel blocker toxicity and, ≥ **One:** ⁽¹⁶⁾

- Calcium
- High dose insulin ⁽¹⁷⁾
- Digoxin toxicity and digoxin-specific antibody ⁽¹⁸⁾
- Isoproterenol ⁽¹⁹⁾
- Permanent pacemaker placement scheduled or performed ≤ 24h ^(13, 14)
- Temporary pacemaker ≤ 2d ⁽²⁰⁾

- Mechanical ventilation

- Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)

● **Episode Day 2, One:**

● **ACUTE, One:** ⁽²¹⁾● **Early responder**, discharge expected today if clinically stable last 12h, **All:** ⁽²²⁾

- Systolic blood pressure within acceptable limits or baseline ⁽²³⁾
- Heart block controlled or resolved
- No complication or active comorbidity relevant to this episode of care ⁽²⁴⁾

● **Partial Responder**, not clinically stable for discharge and requires continued stay, **≥ One:**

- Antiarrhythmic medication adjustment (includes PO) $\leq 2d$ ⁽²⁵⁾

● **Heart failure and, Both:** ⁽²⁶⁾● **Symptoms or findings, ≥ One:** ⁽²⁷⁾

- O_2 sat $\leq 91\%(0.91)$ and $<$ baseline ⁽²⁷⁾
- Worsening edema
- Heart failure confirmed by imaging ⁽²⁸⁾

● **Intervention, ≥ One:** ⁽²⁹⁾● **Diuretic, One:** ⁽³⁰⁾

- $\geq 2x/24h$ (includes PO)
- Requires daily medication adjustment (includes PO) and BUN or creatinine increased from baseline ^(25, 27)
- Dialysis
- Post Critical care $\leq 24h$

● **INTERMEDIATE, One:**● **Early responder**, discharge expected today if clinically stable last 12h, **All:** ⁽²²⁾

- Systolic blood pressure within acceptable limits or baseline ⁽²³⁾
- O_2 sat within acceptable limits ⁽³¹⁾
- Heart block controlled or resolved

● **Partial Responder**, not clinically stable for discharge and requires continued stay, **≥ One:**

- NIPPV ⁽³²⁾
- Oxygen $\geq 40\%(0.40)$, $\leq 2d$ ⁽³³⁾
- Permanent pacemaker placement $\leq 24h$ ^(13, 14)
- Suspected drug toxicity and 2nd degree heart block (Mobitz II) requiring continuous cardiac monitoring (excludes Holter) ^(3, 9)

● **CRITICAL, ≥ One:**

- Hemodynamic monitoring, invasive ⁽¹⁵⁾

● **Junctional rhythm or atrioventricular block, Both:** ^(3, 4)● **Finding, ≥ One:**● **Systolic blood pressure, ≥ One:**

- < 90 mmHg
- < 110 mmHg with chronic hypertension
- > 30 mmHg decrease from baseline ⁽²⁷⁾
- Labile ⁽³⁴⁾

- MAP < 65 mmHg

● **Intervention, ≥ One:**

- Atropine

● **Beta blocker toxicity and, ≥ One:** ⁽¹⁶⁾

- Glucagon
- High dose insulin ⁽¹⁷⁾

● **Calcium channel blocker toxicity and, ≥ One:** ⁽¹⁶⁾

- Calcium
- High dose insulin ⁽¹⁷⁾
- Digoxin toxicity and digoxin-specific antibody ⁽¹⁸⁾
- Isoproterenol ⁽¹⁹⁾
- Permanent pacemaker placement $\leq 24h$ ^(13, 14)
- Temporary pacemaker $\leq 2d$ ⁽²⁰⁾

- Mechanical ventilation

- NIPPV and, **≥ One:** ⁽³²⁾
 - $\text{FiO}_2 > 50\%(0.50)$
 - Respiratory intervention every 1-2h ⁽³⁵⁾
 - Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)
- **Episode Day 3, One:**
 - **ACUTE, One:**
 - **Responder**, discharge expected today if clinically stable last 12h, **All:** ⁽²²⁾
 - Systolic blood pressure within acceptable limits or baseline ⁽²³⁾
 - Heart block controlled or resolved
 - **Complication or comorbidity, One:**
 - No complication or active comorbidity relevant to this episode of care ⁽²⁴⁾
 - Heart failure resolving or at baseline ⁽²⁷⁾
 - **Partial Responder**, not clinically stable for discharge and requires continued stay, **≥ One:**
 - Antiarrhythmic medication adjustment (includes PO) $\leq 2\text{d}$ ⁽²⁵⁾
 - **Heart failure and, Both:** ⁽²⁶⁾
 - **Symptoms or findings, ≥ One:** ⁽²⁷⁾
 - O_2 sat $\leq 91\%(0.91)$ and $<$ baseline ⁽²⁷⁾
 - Worsening edema
 - Heart failure confirmed by imaging ⁽²⁸⁾
 - **Intervention, ≥ One:** ⁽²⁹⁾
 - **Diuretic, One:** ⁽³⁰⁾
 - $\geq 2\text{x}/24\text{h}$ (includes PO)
 - Requires daily medication adjustment (includes PO) and BUN or creatinine increased from baseline ^(25, 27)
 - Dialysis
 - Post Critical care $\leq 24\text{h}$
- **INTERMEDIATE, One:**
 - **Responder**, discharge expected today if clinically stable last 12h, **All:** ⁽²²⁾
 - Systolic blood pressure within acceptable limits or baseline ⁽²³⁾
 - O_2 sat within acceptable limits ⁽³¹⁾
 - Heart block controlled or resolved
 - **Partial Responder**, not clinically stable for discharge and requires continued stay, **≥ One:**
 - NIPPV ⁽³²⁾
 - Oxygen $\geq 40\%(0.40)$, $\leq 2\text{d}$ ⁽³³⁾
 - Permanent pacemaker placement $\leq 24\text{h}$ ^(13, 14)
- **CRITICAL, ≥ One:**
 - Hemodynamic monitoring, invasive ⁽¹⁵⁾
 - **Junctional rhythm or atrioventricular block, Both:** ^(3, 4)
 - **Finding, ≥ One:**
 - **Systolic blood pressure, ≥ One:**
 - < 90 mmHg
 - < 110 mmHg with chronic hypertension
 - > 30 mmHg decrease from baseline ⁽²⁷⁾
 - Labile ⁽³⁴⁾
 - MAP < 65 mmHg
 - **Intervention, ≥ One:**
 - Atropine
 - **Beta blocker toxicity and, ≥ One:** ⁽¹⁶⁾
 - Glucagon
 - High dose insulin ⁽¹⁷⁾
 - **Calcium channel blocker toxicity and, ≥ One:** ⁽¹⁶⁾
 - Calcium
 - High dose insulin ⁽¹⁷⁾

- Digoxin toxicity and digoxin-specific antibody ⁽¹⁸⁾
- Isoproterenol ⁽¹⁹⁾
- Permanent pacemaker placement $\leq 24\text{h}$ ^(13, 14)
- Temporary pacemaker $\leq 2\text{d}$ ⁽²⁰⁾
- Mechanical ventilation
- **NIPPV and, $\geq$ One:** ⁽³²⁾
 - $\text{FiO}_2 > 50\%$ (0.50)
 - Respiratory intervention every 1-2h ⁽³⁵⁾
- Vasopressor, vasodilator, or inotrope, continuous (excludes low or fixed dose for end stage heart failure)
- **Episode Day 4, One:**
 - **ACUTE, One:**
 - **Responder**, discharge expected today if clinically stable last 12h, **All:** ⁽²²⁾
 - Systolic blood pressure within acceptable limits or baseline ⁽²³⁾
 - Heart block controlled or resolved
 - **Complication or comorbidity, One:**
 - No complication or active comorbidity relevant to this episode of care ⁽²⁴⁾
 - Heart failure resolving or at baseline ⁽²⁷⁾
 - **Non-responder**, not clinically stable for discharge and exceeds episode days at the Acute level of care for this condition, **One:**
 - **Use Extended Stay subset**
 - Refer for secondary review
 - **INTERMEDIATE, One:**
 - **Responder**, discharge expected today if clinically stable last 12h, **All:** ⁽²²⁾
 - Systolic blood pressure within acceptable limits or baseline ⁽²³⁾
 - O_2 sat within acceptable limits ⁽³¹⁾
 - Heart block controlled or resolved
 - **Non-responder**, not clinically stable for discharge and exceeds episode days at the Intermediate level of care for this condition, **One:**
 - **Use Extended Stay subset**
 - Refer for secondary review
 - **CRITICAL, One:**
 - **Non-responder**, not clinically stable for discharge and exceeds episode days at the Critical level of care for this condition, **One:**
 - **Use Extended Stay subset**
 - Refer for secondary review

Discharge, Level of Care, One: ⁽³⁶⁾

- **HOME, Both:**
 - **Level of care appropriateness, Both:**
 - Home environment safe and accessible ⁽³⁷⁾
 - Patient or caregiver demonstrates ability to manage care
 - **Arrhythmia discharge planning, All:**
 - **Education on the importance of, All:**
 - Strict medication adherence
 - Participating in an exercise program
 - Avoiding tobacco, excessive alcohol, and illicit drug use
 - Understanding when and where to seek help
 - Provide comprehensive written discharge and teaching instructions ⁽³⁸⁾
 - Medication reconciliation ⁽³⁹⁾
 - Follow-up care planned ⁽⁴⁰⁾
 - Identify and address transportation needs
- **HOME CARE, Both:**
 - **Level of care appropriateness, All:**

- Home environment safe and accessible ⁽³⁷⁾
- Patient or caregiver willing and able to learn care
- Outpatient management contraindicated or unavailable ^(41, 42)
- Skilled services, ≥ **One:** ⁽⁴³⁾
 - Skilled nursing ⁽⁴⁴⁾
 - Skilled therapy, PT, OT, or ST
 - Behavioral health ⁽⁴⁵⁾
- Arrhythmia discharge planning, **All:**
 - Education on the importance of, **All:**
 - Strict medication adherence
 - Participating in an exercise program
 - Avoiding tobacco, excessive alcohol, and illicit drug use
 - Understanding when and where to seek help
 - Provide comprehensive written discharge and teaching instructions ⁽³⁸⁾
 - Medication reconciliation ⁽³⁹⁾
 - Follow-up care planned ⁽⁴⁰⁾
 - Identify and address transportation needs
- **BEHAVIORAL HEALTH, Both:**
 - Level of care appropriateness, **One:**
 - Support systems available ⁽⁴⁶⁾
 - Skilled treatment, ≥ **One:**
 - Clinical assessment ⁽⁴⁷⁾
 - Individual, group, or family counseling
 - Psychiatric, substance, or medication evaluation ⁽⁴⁸⁾
- **SKILLED MEDICAL OR THERAPY, Both:**
 - Level of care appropriateness, **All:**
 - Medical practitioner, NP, PA assessment or oversight ≥ 1x/wk
 - Treatment precluded in a lower level
 - Services, ≥ **One:**
 - Medical and skilled nursing interventions or assessment 1-2x/24h ⁽⁴⁴⁾
 - Therapy, **All:**
 - Goal directed therapy with rehabilitation potential ^(49, 50)
 - Functional impairments requiring at least supervision ⁽⁵¹⁾
 - Able to tolerate 1h/d of skilled therapy ≥ 5d/wk
 - Complete prior to facility transfer, **All:**
 - Medication reconciliation ⁽³⁹⁾
 - Obtain and complete forms for facility
 - Obtain discharge summary and transmit to facility and medical practitioner
 - Arrange transportation
- **SUBACUTE MEDICAL OR THERAPY, Both:**
 - Level of care appropriateness, **All:**
 - Medical practitioner, NP, PA assessment or oversight ≥ 2x/wk
 - Treatment precluded in a lower level
 - Services, ≥ **One:**
 - Medical and skilled nursing interventions or assessment 4h/24h ⁽⁴⁴⁾
 - Therapy, **All:**
 - Goal directed therapy with rehabilitation potential ^(49, 50)
 - ≥ 2 functional impairments requiring at least minimum assistance ⁽⁵¹⁾
 - Able to tolerate 2-3h/d of skilled therapy ≥ 5d/wk and ≥ 1 therapy discipline
 - Complete prior to facility transfer, **All:**
 - Medication reconciliation ⁽³⁹⁾
 - Obtain and complete forms for facility
 - Obtain discharge summary and transmit to facility and medical practitioner

- Arrange transportation

● **SUBACUTE REHAB, Both:**

● Level of care appropriateness, **All:**

- Medical practitioner, NP, PA assessment or oversight $\geq 3x/wk$
- Treatment precluded in a lower level

● Services, **All:**

- Goal directed therapy with rehabilitation potential ^(49, 50)
- ≥ 2 functional impairments requiring at least minimum assistance ⁽⁵¹⁾
- Able to tolerate 2-3h/d of skilled therapy $\geq 5d/wk$ and ≥ 2 therapy disciplines

● Complete prior to facility transfer, **All:**

- Medication reconciliation ⁽³⁹⁾
- Obtain and complete forms for facility
- Obtain discharge summary and transmit to facility and medical practitioner
- Arrange transportation

● **ACUTE REHAB, Both:**

● Level of care appropriateness, **Both:**

- Medical practitioner, NP, PA assessment or oversight $\geq 3x/wk$

● Services, **All:**

- Goal directed therapy with rehabilitation potential ^(49, 50)
- ≥ 2 functional impairments requiring at least minimum assistance ⁽⁵¹⁾
- Able to tolerate $\geq 15h/wk$ of skilled therapy and ≥ 2 therapy disciplines ⁽⁵²⁾

● Complete prior to facility transfer, **All:**

- Medication reconciliation ⁽³⁹⁾
- Obtain and complete forms for facility
- Obtain discharge summary and transmit to facility and medical practitioner
- Arrange transportation

● **LONG TERM ACUTE CARE, Both:**

● Level of care appropriateness, **Both:**

- Treatment precluded in a lower level

● In lieu of acute or continued hospitalization or failed lower level of care, $\geq$ **One:**

- Primary condition or illness and treatment of active comorbid condition(s) ⁽⁵³⁾
- Ventilator dependent $\geq 6h/d$ and weaning planned ^(54, 55)
- Acute and comorbid conditions requiring prolonged hospitalization ⁽⁵⁶⁾

● Complete prior to facility transfer, **All:**

- Medication reconciliation ⁽³⁹⁾
- Obtain and complete forms for facility
- Obtain discharge summary and transmit to facility and medical practitioner
- Arrange transportation

Utilization Benchmarks

Condition or Procedure	% Pd Obs	LOS (days)	Type
2ND DEGREE HEART BLOCK (MOBITZ TYPE II)	18%	2.9	InterQual
308 CARDIAC ARRHYTHMIA AND CONDUCTION DISORDERS WITH MCC	n/a	3.5	CMS GMLOS
309 CARDIAC ARRHYTHMIA AND CONDUCTION DISORDERS WITH CC	n/a	2.3	CMS GMLOS
310 CARDIAC ARRHYTHMIA AND CONDUCTION DISORDERS WITHOUT CC/MCC	n/a	1.8	CMS GMLOS

Condition or Procedure	% Pd Obs	LOS (days)	Type
3RD DEGREE HEART BLOCK	10%	3.1	InterQual
ATRIOVENTRICULAR BLOCK	38%	2.9	InterQual

Percent Paid as Observation indicates the final payment status as a percentage for patients hospitalized with the condition. It may not correspond to the status assigned at admission. Note that patients initially placed in Observation status who are converted to inpatient status (common) are included in the inpatient category, and that patients initially admitted to inpatient status who are converted to observation status (uncommon) are included in the Observation category.

Notes:**1:**

Initial Review criteria are a level of care determination tool, intended to be used as **real-time** decision support in the emergency department to identify if observation or inpatient hospital level services are warranted. They help the reviewer determine whether a patient is appropriate for observation or inpatient admission at the time a decision to admit the patient is being made. Initial Review criteria evaluate **only data that are available at the time the decision is being made**. This may include previously provided interventions or the results of laboratory, imaging, and other tests. Initial Review may be appropriate when bridge or holding orders (e.g., admit to telemetry) are in place. These orders are intended to address the patient's needs until full treatment and medication orders are written.

Initial Review criteria **should not be used** retrospectively or once the decision to admit has been made (e.g., when full treatment and medication orders have been written). If the reviewer has sufficient information to complete an Episode Day 1 review, an Initial Review need not be completed.

Note: While Initial Review enables identification of the level of care, it is not intended to, and should not be used as a substitute for an Episode Day 1 review, which will generally include specific, evidence-based interventions and intensity of service requirements.

For further clarification, see the Acute Level of Care Review Process.

2:

Hemodynamically stable patients who require treatment, assessment, or intervention every 2 to 4 hours.

Note: InterQual® Level of Care criteria are intended to be used to support a patient-specific clinical determination of medical necessity and not for final billing or reimbursement decisions. Final payment decisions consider a variety of factors such as clinical judgment, medical necessity, payer policies and contracts, and state and national regulations.

3:

Heart block is caused by a disturbance in impulse conduction and may be permanent or transient, depending on the cause. Conduction disturbance can be classified by severity into 3 categories:

- First-degree heart block occurs when the PR interval exceeds 0.20 seconds
- Second-degree block includes:
 - Mobitz I (Wenckebach): a progressive PR prolongation until the P wave fails to conduct to the ventricles which results in the QRS complex being dropped. This is followed by normal conduction being restored and a repetition of the sequence.
 - Mobitz II: an intermittent failure of conduction between the atria and ventricles where the PR interval remains constant, but occasionally a P wave is not followed by a QRS complex.
- Third-degree (complete) AV block is present when all the AV impulses fail to conduct to the ventricle and the ventricular escape rhythm is slower than the atrial rate. The site of the block can be the AV node or within the His-Purkinje system.

Treatment of heart block can include atropine, isoproterenol, or a temporary pacemaker. Long-term treatment may require a permanent pacemaker.

4:

Instruction: Patients with hemodynamic instability should be managed at the Critical level of care.

5:

A junctional escape rhythm occurs when the sinoatrial node (SA) or atrioventricular (AV) conduction fail to conduct electrical impulses and the AV node assumes the function of pacemaker for one or more beats. The QRS complex is typically narrow and does not correspond to any preceding P-wave. The heart rate for a junctional escape rhythm is usually 40-60/min. The heart rate for an accelerated junctional escape rhythm is greater than 60/min. Causes of junctional escape rhythms include: sick sinus syndrome, digitalis toxicity, effects of beta blockers or calcium channel blockers (e.g., diltiazem, verapamil), acute myocardial infarction, hypoxemia, and hyperkalemia. Treatment is determined by the cause (e.g., discontinuing a toxic drug or oxygenating the patient). If the heart rate becomes excessively slow, acute treatment with drugs such as atropine or isoproterenol are sometimes required.

Long-term treatment may include permanent pacemaker placement.

6:

Informational: Symptoms of junctional escape rhythm or accelerated junctional escape rhythm are generally limited to dizziness, dyspnea, or presyncope depending on heart rate and blood pressure.

7:

Symptoms include:

- Dizziness
- Dyspnea
- Presyncope
- Syncope

8:

Heart block is caused by a disturbance in impulse conduction and may be permanent or transient, depending on the cause. Second-degree block includes:

- Mobitz I (Wenckebach): a progressive PR prolongation until the P wave fails to conduct to the ventricles which results in the QRS complex being dropped. This is followed by normal conduction being restored and a repetition of the sequence.
- Mobitz II: an intermittent failure of conduction between the atria and ventricles where the PR interval remains constant, but occasionally a P wave is not followed by a QRS complex.

Treatment of heart block can include atropine, isoproterenol, or a temporary pacemaker. Long-term treatment may require a permanent pacemaker.

9:

Instruction: Hospital admission may be appropriate for patients with 2nd degree heart block (Mobitz type II) for diagnostic evaluation and monitoring. Treatment may include evaluation for reversible causes (e.g., medication, ischemia), monitoring, and permanent pacemaker implantation even if asymptomatic.

10:

Hemodynamically unstable patients (or those with the potential to become unstable) who require treatment, assessment, or intervention every 1 to 2 hours.

Note: InterQual® Level of Care criteria are intended to be used to support a patient-specific clinical determination of medical necessity and not for final billing or reimbursement decisions. Final payment decisions consider a variety of factors, such as clinical judgment, medical necessity, payer policies and contracts, and state and national regulations.

11:

Findings of hemodynamic instability include a sustained heart rate > 120/min, systolic blood pressure < 90 mmHg, systolic blood pressure < 110 mmHg with chronic hypertension or, decrease in systolic blood pressure ≥ 30 mmHg from baseline.

12:

Heart block is caused by a disturbance in impulse conduction and may be permanent or transient, depending on the cause. Third-degree (complete) AV block is present when all the AV impulses fail to conduct to the ventricle and the ventricular escape rhythm is slower than the atrial rate. The site of the block can be the AV node or within the His-Purkinje system.

Treatment of heart block can include atropine, isoproterenol, or a temporary pacemaker. Long-term treatment may require a permanent pacemaker.

13:

A cardiac pacemaker is a battery-powered device used to control or maintain a patient's heart rate. It contains a pulse generator that is implanted subcutaneously, usually under the chest wall, and wire electrodes that connect

the generator to the heart chambers being stimulated.

14:

Permanent pacemaker placement is recommended in patients with symptomatic 2nd and 3rd degree heart block (Kusumoto et al., Circulation 2019, 140: e382-e482; Oken et al., JAMA cardiology 2019:).

15:

Invasive hemodynamic monitoring may include at least one of the following methods of assessment: arterial line, pulmonary artery catheter, central venous catheter, or cardiac output monitoring (Rajaram et al., Cochrane Database Syst Rev 2013, 2: CD003408).

16:

For patients experiencing symptoms or hemodynamic instability related to sinus node dysfunction or atrioventricular block caused by beta blocker overdose or calcium channel blocker overdose, guidelines recommend intravenous calcium, glucagon, or high dose insulin therapy to increase heart rate and improve symptoms (Kusumoto et al., Circulation 2019, 140: e382-e482).

17:

Instruction: The administration of high dose insulin in this criteria point refers to the treatment of beta-blocker overdose or calcium channel blocker overdose and is not meant to be used for treatment of hyperglycemia.

18:

For hemodynamically unstable patients, or those with lethal arrhythmia, immunotherapy with digoxin-specific antibodies (known as Fab fragments) may be administered via an intravenous (IV) bolus to bind intravascular free digoxin. A complete response usually occurs within 4 hours. In patients with heart failure, digoxin-specific antibodies should be used with caution. Patients should be monitored for signs of volume overload and hypokalemia due to movement of intravascular potassium into the cell (Kusumoto et al., Circulation 2019, 140: e382-e482).

19:

Isoproterenol should be used with caution in patients with suspected coronary ischemia because it causes an increase in myocardial oxygenation demand (Kusumoto et al., Circulation 2019, 140: e382-e482).

20:

Temporary pacemaker placement is recommended for patients with symptomatic bradycardia or heart block (Kusumoto et al., Circulation 2019, 140: e382-e482; Oken et al., JAMA cardiology 2019:).

21:

Hemodynamically stable patients who require treatment, assessment, or intervention every 4 to 8 hours.

Note: InterQual® Level of Care criteria are intended to be used to support a patient-specific clinical determination of medical necessity and not for final billing or reimbursement decisions. Final payment decisions are based on a variety of factors including medical necessity, payer rules, and contracts.

22:

Selection of this criteria point indicates that the patient is responding to treatment and is clinically stable for transfer or discharge. To determine the most appropriate post-acute level of care, see discharge criteria.

23:

Hypertension is defined as a systolic blood pressure (SBP) of greater than 130 mmHg or a diastolic blood pressure (DBP) of 80 mmHg or greater. Recommendations for initial management may include a conservative approach with diet and lifestyle modification with or without medications. The goal of antihypertensive therapy is to reduce morbidity and mortality. In non-urgent situations, blood pressure (BP) control is achieved in the outpatient setting. Goals for target BP level should be individualized, but a SBP less than 130 mmHg and a DBP less than 80 mmHg are

desirable. Populations at high risk include patients with the following conditions (Whelton et al., Hypertension 2018, 71: 1269-324):

- Clinical cardiovascular disease or 10-year atherosclerotic cardiovascular disease (ASCVD) risk 10%(0.1)
- Heart failure
- Stable ischemic heart disease
- Chronic kidney disease
- Chronic kidney disease after renal transplantation

24:

Relevant complications or comorbidities are conditions that require active management during an inpatient stay.

25:

Instruction: For these criteria, medication adjustment refers to a change in dose, frequency of administration, or change in medication.

26:

Management of patients admitted with a heart failure (HF) exacerbation may include supplemental oxygen, diuretic, guideline-directed medical therapy (GDMT), which includes angiotensin-converting enzyme inhibitor (ACEi) or angiotensin receptor blocker (ARB) or angiotensin receptor neprilysin inhibitor (ARNi), beta-blocker, mineralocorticoid receptor antagonists (MRA), and sodium-glucose cotransporter 2 inhibitors (SGLT2i), cardiac monitoring, and deep vein thrombosis (DVT) prophylaxis (Maddox et al., J Am Coll Cardiol 2021, 77: 772-810).

27:

Baseline refers to either the patient's normal baseline or a newly established baseline. In the absence of documentation, a patient's baseline status may be presumed to be normal.

28:

Bilateral pulmonary infiltrates seen on chest x-ray are consistent with the diagnosis of left-sided heart failure.

29:

In patients hospitalized with acute heart failure (HF) due to an increased risk of atrial and ventricular arrhythmias, continuous cardiac monitoring is recommended. Monitoring may be discontinued when there is no evidence of a hemodynamically significant arrhythmia and the symptoms of acute HF have improved (Sandau et al., Circulation 2017, 136: e273-e344).

30:

Intravenous (IV) loop diuretics (e.g., furosemide, torsemide, bumetanide, ethacrynic acid) are administered to relieve the symptoms of dyspnea and congestion without excessively reducing intravascular volume. Due to their relatively short half-life, diuretic effectiveness can be enhanced by continuous administration or multiple boluses daily. Careful monitoring of daily weights, orthostatic vital signs, intake and output, electrolytes, and renal function are key components of diuretic therapy. If a patient does not initially respond to IV diuretics, other options may be considered, including increasing the dose to ensure adequate drug levels reach the kidneys and adding a second diuretic, typically a thiazide, to the loop diuretic in order to improve diuretic responsiveness (Heidenreich et al., Circulation 2022, 145: e895-e1032; Maddox et al., J Am Coll Cardiol 2021, 77: 772-810; Rosendorff et al., Circulation 2015).

31:

When a criteria point states "within acceptable limits," it refers to either the patient's normal baseline, a newly established baseline, or parameters that the medical practitioner determines are acceptable.

32:

Noninvasive positive pressure ventilation (NIPPV), also known as NIV, provides respiratory support by application of a tightly fitting facial mask, nasal mask, or helmet rather than an endotracheal tube or tracheostomy. In some

cases, the use of NIPPV can avoid the need for endotracheal intubation and decreases the risk of barotrauma, lung injury, and/or infection. NIPPV is commonly delivered by a bilevel positive airway pressure ventilator (BiPAP), a continuous positive airway pressure device (CPAP), or a mechanical ventilator. Supplemental oxygen can be delivered at concentrations approximating 100%(1.0).

The decision to provide respiratory support via NIPPV, and the modality to provide NIPPV, is based upon the patient's specific clinical findings (e.g., medical condition leading to respiratory failure, underlying comorbidities, clinical progression, improvement). Examples of NIPPV devices are volumetric (i.e., deliver a defined volume), barometric (i.e., deliver a defined pressure), and combined (i.e., deliver defined volumes and pressure). The terminology for NIPPV delivery systems may differ between equipment manufacturers and provider organizations. InterQual® criteria do not differentiate between the different NIPPV modalities.

Whether or not high-flow nasal cannula (HFNC) should be classified as a component of NIPPV is unclear, and there is conflicting evidence in the medical literature. Where HFNC is an appropriate modality, InterQual® defines this in a specific criteria point. As such, NIPPV does not include HFNC (Hackett, A., PulmCCM 2018; Nardi et al., F1000Res 2017, 6: 290; Osadnik et al., Cochrane Database Syst Rev 2017, 7: CD004104; Allison and Winters, Emerg Med Clin North Am 2016, 34: 51-62; Gregoretti et al., Crit Care Clin 2015, 31: 435-57).

33:

Oxygen therapy is the administration of oxygen at concentrations greater than ambient air (room air: 21%(0.21)) with the intent of treating and/or preventing the symptoms and manifestations of hypoxia. The oxygen concentration or percentage (FiO₂) delivered varies with the manufacturer's design, oxygen flow rate, the patient's respiratory rate, and tidal volume. Actual inspired FiO₂ with nasal cannula varies significantly with the patient's minute ventilation and pattern of breathing. For patients at risk for CO₂ retention, where a precise inspired FiO₂ is required, the Venturi mask is the preferred method. In general, patients who are very ill or have respiratory disease may require considerably higher flow rates to achieve the desired FiO₂. The following are estimates of O₂ delivered at the associated flow rates:

LOW-FLOW SYSTEMS

Nasal cannula

Room air 21%(0.21)

1L 24%(0.24) 4L 36%(0.36)

2L 28%(0.28) 5L 40%(0.40)

3L 32%(0.32) 6L 44%(0.44)

Simple oxygen masks

- 35-50%(0.35-0.50) FiO₂ (5-10 L/min)
- Flow rates are usually maintained at 5 L/min or more to avoid accumulation of CO₂ in the mask

Venturi masks

- 24-50%(0.24-0.50) FiO₂ (4-12 L/min)

Partial rebreathing masks

- 40-70%(0.40-0.70) FiO₂ (6-10 L/min)

Non-rebreathing masks

- 60-80%(0.60-0.80) FiO₂ (minimum flow of 10 L/min)

HIGH-FLOW SYSTEMS

Air-entrainment masks/nebulizers

- 24-40%(0.24-0.40) FiO₂

Heated, humidified high-flow concentrating nasal cannula (HFNC)

- 24-100%(0.24-1.0) FiO₂

34:

Blood pressure is considered labile when it is extremely variable and difficult to control.

35:

Respiratory interventions include ventilator/noninvasive positive pressure ventilation (NIPPV) setting changes, chest physiotherapy, suctioning, and nebulizer or metered-dose inhaler (MDI) treatments.

36:

The discharge screen is a resource tool and not criteria. Referring to the discharge screen at the initiation of discharge planning is recommended.

37:

The home environment and safety assessment will normally include an evaluation of the patient's pre- and post-hospitalization functional level, physical layout of the home (e.g., entrances and exits, stairs, access to the community), identification of unsafe conditions (e.g., scatter rugs, missing handrails, oxygen use and smoking, lack of fire safety devices, inadequate lighting, heating, and cooling), factors that may trigger symptoms (e.g., secondhand smoke, poor food choices, dysfunctional family dynamics), sanitation hazards (e.g., lack of electricity, running water, refrigeration, inadequate toilet facilities, presence of insects or rodents), and the need for adaptive equipment (e.g., walker, handrails for the tub or shower, elevated toilet seat).

38:

Ensuring the patient and/or caregiver understands all aspects of the condition and can assume responsibility for self-care is crucial in preventing readmission. Assessing a patient and/or caregiver's level of understanding after education has been provided can be difficult. The teach-back method is an effective way of providing education at the appropriate level and can also be used to assess the learner's comprehension. To use the teach-back method, discuss key points in common terms and avoid using medical jargon or unfamiliar terms. After the education has been delivered, ask the learner to repeat what was learned. Gaps or misinterpretations in the learner's explanation will pinpoint areas where communication may have failed and provide opportunity for clarification.

39:

Medication reconciliation is a formal process or technique used by health care providers and pharmacists to identify the most complete and accurate list of all medications a patient is taking at times of transitions in care (e.g., upon hospital admission, transfer from one unit to another during hospitalization, or discharge from the hospital to home or another facility). The goals of this process are to ensure medication and dosages are appropriate for the patient, resolve discrepancies in drug regimens, and ultimately prevent medication errors and reduce adverse drug events. Medication reconciliation is a Joint Commission National Patient Safety goal. Coordinating information when a patient is transferred to another setting, service, practitioner, or level of care ensures accurate medications are listed. The process is comprised of the following (Hospital: National Patient Safety Goals. 2020):

- Obtain an external list of medications (e.g., medications taken prior to admission)
- Develop a list of current medications and add them to the medical record
- Compare the medications from the external list to the current list
- Clarify inconsistencies/discrepancies (e.g., omissions, duplications, contraindications, unclear information, and changes)
- Develop a list of medications to be prescribed at discharge or transfer
- Communicate the new list to the patient and/or appropriate caregiver(s)
- Ensure that patient and/or caregiver(s) understand the medication information upon discharge

Specific medication issues identified as being problematic include missing medication information from transfer orders, lack of information on medications provided in the acute setting, incomplete medication records, discrepancies between hospital regimen and discharge summary, and missing information pertaining to the patient's tolerance of a medication regimen. Although medication reconciliation is an important aspect in patient safety, there is a lack of consensus and evidence about the best effective methods of implementing this process. Physician-led and electronic medication reconciliation in hospitals are effective strategies to reduce medication discrepancies, however, the impact of these interventions is uncertain due to the low quality of evidence (Choi and Kim, J Clin Pharm Ther 2019, 44: 932-45; Redmond et al., Cochrane Database Syst Rev 2018, 8: CD010791). Trained pharmacy technicians, under the direction of a licensed pharmacist, may be an option for developing and expanding medication reconciliation processes (Irwin et al., Hosp Pharm 2017, 52: 44-53).

40:

Close follow-up of older adults, pediatric, or at-risk patients after discharge can help minimize hospital readmission and total health care costs (Rising et al., Ann Emerg Med 2013, 62: 145-50; Frei-Jones et al., Pediatr Blood Cancer 2009; 52(4): 481-485; Kripalani et al., JAMA 2007; 297(8): 831-841). Studies have shown that for greater than 50% (0.50) of patients re-hospitalized within 30 days of discharge, there was no visit to a physician's office between the

time of discharge and re-hospitalization (Hernandez et al., JAMA 2010, 303: 1716-22; Jencks et al., N Engl J Med 2009; 360(14): 1418-1428). Efforts to reduce readmissions should focus on patients known to be at high risk and patients who have frequent visits to the emergency department (ED). Patients at high risk may have multiple ED visits prior to readmission (Rising et al., Ann Emerg Med 2013, 62: 145-50).

41:

Outpatient management contraindicated refers to the following:

- The patient whose medical condition is so fragile that going to the clinic or medical practitioner's office for treatment would put the patient at risk
- The patient with extensive equipment (e.g., ventilators, specialized beds) where travel outside the home would be cumbersome and places the patient at risk
- The treatment goal is to adapt the home environment for increased independence (e.g., an individual who is blind who needs mobility education)

42:

Outpatient management unavailable refers to the following:

- The patient whose treatment (e.g., dressing changes) is so extensive that it takes too long or is too frequent (e.g., twice daily) to be done in an outpatient clinic or medical practitioner's office
- The scheduling of the treatment falls outside the medical practitioner's office or clinic hours
- Travel distance to receive care is excessive (e.g., greater than 1 hour)
- The service is unavailable in the outpatient setting

43:

Skilled nursing services refer to services that must be provided by a licensed nurse qualified to assess and monitor patient condition(s) and provide medical treatment and/or teaching to patients who have skilled nursing needs. Examples of Home Care level of care interventions include skilled assessment, intervention, or education for any of the following:

- New treatment or medication regimen
- Adjustment in the treatment or medication regimen
- Infusion therapy administration, teaching, or access device management
- Management and evaluation of a care plan for patients with an active comorbidity and multiple care needs
- Chronic disease requiring a disease management program (e.g., asthma, heart failure, chronic obstructive pulmonary disease, diabetes)
- End-stage disease, hospice, or palliative care

44:

Skilled nursing interventions refer to services that are provided or supervised by a qualified or licensed professional when the care needs of the patient require assessment, observation, monitoring, and/or education to achieve the medically desired result. Close observation and assessment by nursing may be necessary at this level of care. This includes nursing care such as medication administration (excludes routine), vital signs monitoring, and/or other interventions not listed.

45:

The Home Care criteria are used for a patient who has been admitted or is expected to be admitted for home care. When criteria are met for this level of care and it is not available in your area, we recommend you use the next higher level of care.

Home care can include specialty services provided by a psychiatric nurse following a specialized treatment plan developed by a psychiatrist or advanced practice registered nurse. It is intended for treatment and monitoring of a patient's psychiatric condition that prevents the patient from leaving the house or makes leaving the house extremely difficult or unsafe (Centers for Medicare & Medicaid Services. Chapter 7, In: Medicare benefit policy manual. 2022). Home care can also be used for a patient with severe and persistent mental illness with a history of nonadherence to medication.

Evaluation and Treatment

Programming may differ based upon legislative and geographical variances and is subject to organizational policy; however, at a minimum, it should include:

- Care coordination with treating physician and other providers
- Clinical assessment every visit
- Education every visit
- Medication assessment, teaching, and management every visit
- Medication reconciliation initiated within first visit
- Psychosocial assessment within first visit
- Substance evaluation within first 2 visits

For those patients who are covered under a Medicare benefit, the Affordable Care Act mandates that a certifying physician or allowed practitioner have a face-to-face visit with the patient within 90 days prior to the start of care, or within 30 days after the start of care. There must be documentation of the patient's condition that supports the need for home care services and the requirement of homebound status (Centers for Medicare & Medicaid Services. Chapter 7, In: Medicare benefit policy manual. 2022).

46:

Support system includes friends, family, social services, health care delivery providers or agencies, sponsors, clergy, and self-help groups.

47:

The initial clinical assessment is performed by a licensed professional and includes a comprehensive review of the patient's presenting diagnosis and a review of body systems. Identification of current and potential medical needs and health problems can be identified by the clinical assessment. The clinical assessment includes:

- History and physical exam (e.g., vital signs, height, weight)
- Pain assessment includes cause, intensity, quality, onset, duration, and effects on quality of life (e.g., activities of daily living (ADLs), instrumental activities of daily living (IADLs), and interpersonal relationships). Pain relievers should also be included
- Nutritional and hydration status
- Functional ability
- Safety and infection control measures
- Prescribed and over-the-counter (OTC) medications, herbal supplements, and home remedies
- Patient's and/or caregiver's understanding of medication use, dosing, side effects, and adherence to the medication or treatment regimen
- Patient's and/or caregiver's understanding of the illness or disease process and potential long-term complications
- Patient's and/or caregiver's coping strategies to deal with the illness or injury and ability to follow the plan of care

48:

For specific level of care recommendations for the treatment of psychiatric and substance use disorders, refer to the InterQual® Adult and Geriatric Psychiatry, InterQual® Child and Adolescent Psychiatry, or InterQual® Substance Use Disorders products.

49:

Goal-directed therapy includes one or more of the following:

- Activities of daily living (ADLs) (e.g., feeding, dressing, bathing, grooming, toileting, functional mobility)
- Bed mobility or transfers (e.g., transfers to chair, toilet, commode, tub, shower, car)
- Cognitive, speech, language, or swallowing retraining
- Home and lifestyle modification (e.g., assessment and training focused on the home, job, school, or work environment)
- Pain management techniques (e.g., assessment and interventions used to relieve pain or decrease its intensity)
- Pulmonary rehabilitation (e.g., energy conservation, speech, breathing re-education, postural drainage, percussion, or vibration)
- Positioning and splinting
- Range of motion and strengthening
- Wheelchair mobility
- Ambulation

50:

Rehabilitation potential refers to the probability that therapy and medical goals are realistic and attainable based on patient's prior level of function, severity of illness or injury, and the extent of impairments.

51:

Functional assistance levels are based upon the patient's function during tasks and activities necessary to return to household mobility or ambulation. The functional assistance level required for each individual task or activity (e.g., mobility, activities of daily living (ADLs)) may vary.

The following terms are commonly used in the post-acute setting:

- Independent - Patient can safely and within a reasonable amount of time perform a task (or developmentally appropriate task) without physical or cognitive assistance or supervision
- Modified Independent - Patient performs an activity with a supportive device, adaptive equipment, and/or prosthetic or orthotic device. Additional time may be required to complete the activity and/or there are safety (risk) considerations
- Supervision - Patient performs an activity with standby or distant supervision or setup. Verbal cueing or coaxing, without physical contact or setup of items and application of orthoses may be required when patient's safety awareness is impaired
- Minimum or limited Assistance - Patient performs at least 75%(0.75) of an activity and requires some physical contact to steady, guide, or move
- Moderate or extensive Assistance - Patient performs at least 50%(0.50) of an activity and requires physical assistance for functional mobility or ADLs
- Maximum Assistance - Patient performs 25%(0.25) to 50%(0.50) of an activity and requires physical assistance for functional mobility or ADLs
- Total Assistance or dependence - Patient performs less than 25%(0.25) of an activity and may require total assistance for functional mobility or ADLs

52:

The intensity of rehabilitation therapies is a driver for admission to an acute inpatient rehabilitation facility. Documentation should reflect the need for required and ongoing intensive therapies. The determination of whether a patient requires and can tolerate an intensive rehabilitation program of at least 15 hours is not based on any one single piece of documentation. All clinical documentation should be evaluated by the reviewer to determine whether the patient is demonstrating the need for services and has the ability to tolerate them. Patients who do not require this level of intensive therapy should be treated at a lower level of care. Rehabilitation therapies should be reasonable and medically appropriate and should not exceed a patient's level of tolerance. If there are clinical features which preclude the patient's ability to tolerate the intensity threshold of at least 15 hours per week, secondary review would be appropriate (Centers for Medicare & Medicaid Services, Medicare Benefit Policy Manual Chapter 1 - Inpatient Hospital Services Covered Under Part A. 2021; Miller and Forer, Pm r 2021, 13: 535-9; Forrest et al., Medicine (Baltimore) 2019, 98: e17096).

53:

Comorbid conditions include new onset or worsening behavioral symptoms, heart failure, deep vein thrombosis, uncontrolled diabetes, immunocompromised states, malignant or end-stage disease, malnutrition, renal insufficiency or end-stage renal disease, infection, ventilator dependence, noninvasive positive pressure ventilation or respiratory insufficiency, complex wound care, and new functional impairment(s) with limitation.

54:

Ventilation includes invasive ventilation and noninvasive positive pressure ventilation (NIPPV), including continuous positive airway pressure (CPAP) and bilevel positive airway pressure (BiPAP).

55:

Notable causes for prolonged mechanical ventilation include:

- Ventilator-acquired pneumonia
- Renal failure
- Heart failure
- Exacerbation of chronic obstructive pulmonary disease (COPD)

- Chest wall disorder
- Neuromuscular diseases
- Traumatic brain injury
- Sepsis
- Complications following cardiac surgical procedures

Major contributing factors for failure to wean include (Ambrosino and Vitacca, Multidiscip Respir Med 2018, 13: 6; Davies et al., Br J Anaesth 2017, 118: 563-9):

- Increased work of breathing
- Impaired respiratory drive
- Inspiratory muscle weakness

56:

Long-Term Acute Care (LTAC) is a recognized level of care designation by the Centers for Medicare and Medicaid Services (CMS) for acute care hospitals. It is defined by federal statutes as a level of care for patients requiring an average length of stay of 25 days or more (Centers for and Medicaid Services, Fed Regist 2018, 83: 41144-784). Patients who require a short stay for treatment may be more appropriate for the subacute or skilled nursing facility (SNF) level of care.